BEMETHA TABLET 0.5MG

DESCRIPTION

An 8 mm diameter, round, flat, scored, blue tablet.
Each tablet contains Betamethasone 0.5 mg.

INDICATION

Suppression of inflammatory and allergic disorders; congenital adrenal hyperplasia; cerebral oedema.

PHARMACODYNAMICS

Betamethasone diffuses across cell membranes and complex with specific cytoplasmic receptors which then enter the cell nucleus, bind to DNA, and stimulate transcription of messenger RNA (mRNA) and subsequently protein synthesis of various enzymes thought to be ultimately responsible for glucocorticoid effects of systemic corticosteroids. Betamethasone decreases or prevents tissue responses to inflammatory processes, thereby reducing development of symptoms of inflammation without affecting the underlying cause. Betamethasone inhibits accumulation of inflammatory cells, including macrophages and leukocytes, at sites of inflammation. It also inhibits phagocytosis, lysosomal enzyme release, and synthesis and/or release of several chemical mediators of inflammation. Mechanisms of immunosuppressant action are not completely understood but may involve prevention or suppression of cell-mediated (delayed hypersensitivity) immune reactions as well as more specific actions affecting the immune response. Betamethasone also inhibits corticotropin secretion, leading to suppression of adrenal hypersecretion of androgens responsible for the androgenism associated with various enzyme deficiencies.

PHARMACOKINETICS

Betamethasone is rapidly and almost completely absorbed from the gastrointestinal tract. It is rapidly distributed to all body tissues. It crosses the placenta and may be excreted in small amounts in breast milk. Most betamethasone in the circulation is extensively bound to plasma proteins, mainly to globulin and less so to albumin. Betamethasone is primarily metabolised in the liver but also in the kidney and tissue; mostly to inactive metabolites, which are then excreted renally. Peak effects occur 1-2 hours following oral administration. It has a plasma half-life of 3-5 hours but its duration of action depends upon the biological (tissue) half-life, which ranges from 36-54 hours. Therefore, its effects may last for up to 3.25 days.

DOSAGE

Adult
0.5 -7 mg daily as a single dose or in divided doses.

SIDE EFFECT

Increased appetite, indigestion, nervousness or restlessness and trouble in sleeping occur frequently. Cataracts, diabetes mellitus, changes in skin colour or hypopigmentation, dizziness or lightheadedness, flushing of face or cheeks and unusual increase in hair growth on body or face have been reported.

Long-term use : Acne or other skin problems, avascular necrosis, Cushing's syndrome, edema, endocrine imbalance, gastro intestinal irritation, hypokalaemic syndrome, osteoporosis or bone fractures, including vertebral compression and long bone pathologic fractures, pancreatitis, peptic ulceration or intestinal perforation, steroid myopathy, striae, unusual bruising and impaired wound healing.

Withdrawal syndrome : Sudden withdrawal or reduction in dosage may precipitate acute adrenal insufficiency. Symptoms include malaise, muscle weakness, muscle or joint pain, abdominal or back pain, dizziness, fainting, low-grade fever, prolonged loss of appetite, nausea, vomiting, shortness of breath, frequent or continuing unexplained headaches, unusual tiredness or weakness, rapid weight loss, hypoglycaemia, hypotension, and dehydration. Reappearance of disease may also occur.

CONTRAINDICATIONS

Patients hypersensitivity reactions to betamethasone or other corticosteroids, or to any of the components of Bemetha Tablet 0.5mg. Betamethasone is contraindicated in the presence of acute infections because of interference with inflammatory and immunological responses.

PRECAUTIONS

Caution is recommended in patients with recent intestinal anastomoses, cardiac disease, congestive heart failure, hypertension or renal function impairment. Betamethasone may also exacerbate diabetes mellitus, systemic fungal infections, open-angle glaucoma and osteoporosis. Patients with hepatic function impairment, hypoalbuminaemia, hepatic cirrhosis and nephrotic syndrome may be at increased risk of betamethasone toxicity. Betamethasone should also be used with care in patients with ocular herpes simplex, oral herpetic lesions, hyperlipidaemia, hyper- and hypothyroidism. Betamethasone may increase risk of aseptic necrosis in patients with systemic lupus erythematosus and may exacerbate or reactivate active and latent tuberculosis.
Geriatrics: Geriatric patients may be more likely to develop hypertension during betamethasone therapy. Postmenopausal women, may also be more likely to develop glucocorticoid-induced osteoporosis.

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17.1cm (w) x 16cm (h)

Black 100%

Pheochromocytoma crisis, which may be fatal, has been reported after administration of systemic corticosteroids. Corticosteroids should only be administered to patients with suspected or identified pheochromocytoma after an appropriate risk/benefit evaluation.

DRUG INTERACTIONS

Estrogens may increase the therapeutic and/or toxic effects of betamethasone. Concurrent use with digitalis glycosides may increase possibility of arrhythmias or digitalis toxicity. Concurrent administration of barbiturates, carbamazepine, phenytoin, primidone or rifampicin may enhance the metabolism and reduce the effects of betamethasone. Risk of gastrointestinal ulceration or haemorrhage may be increased when alcohol, nonsteroidal anti-inflammatory drugs (NSAIDs) or anticoagulants are used concurrently with betamethasone. Concurrent use with ephedrine, folic acid, mexiletine, ritodrine, salicylates and streptozocin is also not recommended.
Co-treatment with CYP3A inhibitors, including cobicistat-containing products, is expected to increase the risk of systemic side-effects. The combination should be avoided unless the benefit outweighs the increased risk of systemic corticosteroid side-effects, in which case patients should be monitored for systemic corticosteroid side-effects.

OVERDOSAGE

Acute overdosage is not expected to lead to a life-threatening situation. Acute overdosage should be treated by inducing emesis or by the administration of gastric lavage.

Up-to-date information on treatment of overdose can be obtained from The National Poison Centre, Universiti Sains Malaysia.

TOXICOLOGY

Pregnancy/Reproduction
Fertility : Betamethasone has been reported to increase or decrease the number or motility of spermatozoa.

Pregnancy : Betamethasone crosses the placenta. Infants born to mothers who have received substantial doses of betamethasone during pregnancy should be carefully observed for signs of hypoadrenalism and replacement therapy administered as required. Physiologic replacement doses of betamethasone administered for treatment of maternal adrenal insufficiency are also unlikely to adversely affect the foetus or neonate.

Breast –feeding : The use of higher pharmacologic doses is not recommended because betamethasone is excreted in breast milk and may cause growth suppression and inhibition of endogenous steroid production, in the infant.

PRESENTATION

Blister pack : 100 x 10's

STORAGE CONDITIONS AND USER INSTRUCTIONS

Store in a dry place below 30°C.
Protect from light.
Keep out of reach of children.
Jauhi daripada kanak-kanak.
Shelf life : Please refer to outer package.
Do not use more medicine than the amount recommended.
Take medicine with meals to lessen gastrointestinal irritation.

Route of administration: Oral

Product Registration Holder:
Duopharma Manufacturing (Bangi) Sdn. Bhd.
Lot No. 2, 4, 6, 8, & 10, Jalan P/7, Section 13,
Bangi Industrial Estate, 43650 Bandar Baru Bangi,
Selangor, Malaysia.

Manufacturer:
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DUOPHARMA

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